



WA Advance Directives Acknowledgement

Health Care directives (Advance Directives) are instructions written for health care providers, in advance of the need for medical treatment. You are not required to have any Health Care Directives to live at Merrill Gardens. Some common examples are listed below. It is a good idea to give copies of your Directives to us, and to other Health Care Providers. In an emergency, we will provide copies of Directives we have on file to hospitals, paramedics, etc.

Please check which Directives are currently in effect for you and give us a copy of the document(s).

- ☐ Power of Attorney for Finances
- ☐ Power of Attorney for Health Care
- ☐ Health Care Directive, Advance Directive or Living Will
- ☐ POLST form (sometimes called No Code, Do Not Resuscitate, or DNR)
- ☐ Anatomical Gifts directives
- ☐ Other documents _____

CPR and First Aid: If you are not breathing or have no pulse, we will call 911. However, if we have a properly completed POLST form for you that indicates "No CPR", we will not initiate CPR. In the absence of a valid POLST form, we will begin CPR.

If this occurs during a witnessed choking incident or other accident, we will begin CPR and provide basic first aid. If your POLST form states "No Exceptions" to the DNR order, we will not start CPR if you are not breathing or have no pulse, regardless of the circumstances. If you are receiving Hospice services, we can make the exception described below.

Hospice Exception: If a resident receiving Hospice Services has a completed POLST form, and a written hospice plan that gives staff instructions on what to do if the resident stops breathing or his/her heart stops beating, the community staff may withhold CPR, and will not have to call "911." We will call Hospice.

Where do we keep your POLST and other Advance Directives? Put the documents (original if a POLST form) on your refrigerator door. We will also keep copies in your health chart or at the front desk. We can provide you with a sample POLST form, which is recommended by the Department of Health, and should be honored by Emergency Medical Personnel. Your Doctor must complete this form with you.

Merrill Gardens employees cannot notarize or sign as a witness for any financial or health care documents, as it may pose a conflict of interest.

I acknowledge that I have read the above, or it has been read to me. I understand Merrill Gardens' policies on Directives and Powers of Attorney. I understand that Merrill Gardens will perform CPR and first aid, regardless of my directive(s) if a licensed nurse is not present. I have also received the Aging and Disability Services Administration (ADSA) information entitled *Fact Sheet-Advance Directives and Informed Consent*.

Resident/Responsible Party Signature

Date

FACT SHEET

ADVANCE DIRECTIVES AND INFORMED CONSENT

Health care decision-making issues involving topics such as advance directives and informed consent are both sensitive and complicated. Every situation must be reviewed on a case by case basis. Below are some general concepts that apply to issues that may arise in any setting, including an in-home setting.

ADVANCE DIRECTIVES

WHAT IS AN ADVANCE DIRECTIVE?

An advance directive is a written document that describes an individual's choice about how health care decisions should be made in the future if the individual is unable to provide that information. Examples of advance directives include durable power of attorney for health care and living wills. They often address end-of-life issues. The federal Patient Self Determination Act requires most facilities/homes to provide information to residents about advance directives. The Act also gives each resident the right to choose whether or not to make an advance directive.

WHO HAS TO PROVIDE INFORMATION TO A RESIDENT ABOUT ADVANCE DIRECTIVES?

The federal Patient Self Determination Act requires all health care providers who participate in the Medicare or Medicaid program to provide information to residents about advance directives, and to give them an opportunity to execute advance directives if they choose to do so.

WHEN SHOULD BE DONE BEFORE IMPLEMENTING AN ADVANCE DIRECTIVE?

Even when the resident's current medical event is addressed by the resident's advance directive, the provider and staff must first inform the resident or the resident's legal decision maker about possible treatment before implementation of the advance directive.

WHAT SHOULD I DO IN AN EMERGENCY IF I CANNOT OBTAIN AN INFORMED CONSENT DECISION?

RCW 7.70.050 allows a provider to implement an advance directive in good faith if, in an emergency, the provider cannot obtain an informed consent decision due to the incompetence of the resident or the unavailability of the legal decision maker. Because some assessment and decisions about end-of-life issues may only be made by a licensed practitioner, please contact your attorney for more specific information, including any questions about scope of practice issues.

IS THERE A SPECIFIC FORM REQUIRED FOR ADVANCE DIRECTIVES?

No. A specific form is not required for advance directives. There are forms available that you may access and use but no specific form is required.

WHAT ARE MY RESPONSIBILITIES AS A LONG TERM CARE FACILITY/HOME ON HEALTH CARE DECISION MAKING ISSUES?

All long-term care facilities/homes must have operational policies and procedures directing staff how to handle a resident's medical emergency, including informed consent and implementation of advance directives. This also includes the use of the POLST form if the resident has one.

ARE THERE ANY SPECIFIC ASSESSMENT RELATED ISSUES THAT I NEED TO CONSIDER?

All long-term care facilities/homes must assess resident's cognitive capacity and identify whether the resident or someone else has the authority to make health care decisions for the individual. Part of this process would involve 1) determining whether the resident has executed an advance directive, and if the advance directive is in effect, and 2) if the resident will not be making his or her own decisions, identifying the appropriate surrogate decision maker. This information should be documented in the resident's record.

FACT SHEET

ADVANCE DIRECTIVES AND INFORMED CONSENT

SHOULD ADVANCE DIRECTIVES AND ANY POLST FORM BE ACCESSIBLE?

Yes. Both should be kept in a place easily accessible by anyone who has the right to or need for that information. The Washington State Medical Association recommends the POLST form being the first document in the resident's record. If this type of document is kept in the same place for each resident, then staff or anyone else who has the right to the information will be able to quickly access the information. For example, the facility/home might decide that residents' POLST form will be the top document in each resident's file.

CAN I BE A POWER OF ATTORNEY FOR A RESIDENT?

Owners, operators, administrators and employees of a long term care facility/home where a resident resides cannot act as power of attorney for the resident unless they are also the spouse, adult child or brother or sister of the resident. (RCW 11.94.010)

INFORMED CONSENT

WHAT IS INFORMED CONSENT AND WHO MUST BE GIVEN INFORMED CONSENT?

The law (chapter 7.70 RCW) provides everyone with the right to be fully informed of health care issues that have the potential to affect their lives. As part of the informed consent process, the health care provider must tell individuals about the risks and benefits of certain actions or treatments, and about the risk of not having treatment. Actions or treatments that require informed consent include decisions about what medication to take, whether to change medical treatment, or how to handle life threatening problems. Except in an emergency, informed consent must be given before a health care decision is made, whether or not the person has an advance directive.

WHO IS AUTHORIZED TO PROVIDE THE INFORMATION A PERSON NEEDS IN ORDER TO MAKE AN INFORMED DECISION?

Usually the information should be provided by the practitioner who orders the medication or treatment in question. RCW 7.70.020 gives health care providers the authority to give the information. Health care providers are defined as persons, including physicians and nurses, licensed by this state to provide health care or related services. If the health care provider's license gives him/her the scope of practice to provide informed consent information, then he or she may do so.

WHO IS AUTHORIZED TO MAKE AN INFORMED CONSENT DECISION FOR HEALTH CARE WHEN A RESIDENT IS NOT COMPETENT?

The state law regarding informed consent (RCW 7.70.065) lists, in the order of priority, the persons authorized to provide informed consent for health care on behalf of individuals who are not competent to consent themselves. Long term care facilities and other health care providers are required to use this list to determine the appropriate surrogate decision maker.

- The appointed guardian of the patient, if any;
- The individual, if any, to whom the patient has given a durable power of attorney that encompasses the authority to make health care decisions (DPOAHC);
- The patient's spouse;
- Children of the patient who are at least eighteen years of age;
- Parents of the patient; and
- Adult brothers and sisters of the patient.

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ADVANCE DIRECTIVES AND INFORMED CONSENT

MAY NIECES OR NEPHEWS MAKE INFORMED CONSENT DECISIONS FOR AN INDIVIDUAL?

Since nieces and nephews are not on the list of surrogate decisions makers, they may only make informed consent decisions if they have been appointed guardian for the individual, or if the individual has designated them as durable power of attorney for health care decisions.

WA Death with Dignity

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

It is the policy of this community to allow residents the freedom to exercise their right under the Washington Death with Dignity Act (RCW 70.25) (the Act). The process will be conducted in accordance with applicable state statutes, regulations and community policies. However, at no time, in any part of the process, will the community or its team members actively participate in the process, including but not limited to:

- Serving as a witness to any written request by a resident for medication that the resident may self-administer to end his or her life under the Act;
- Attending or otherwise being present during and after the injection or ingestion of a lethal dose of medication, except in the delivery of comfort care services and outlined in the resident's service plan;
- Taking possession of the medication for storage or delivery.

Guidelines

Procedure

Upon awareness of a resident's intent to exercise their right to request a medication for the purpose of ending his or her life the community will proceed as follows:

1. If a resident is making a written request for medication under the Act, the Act requires that they must have at least two witnesses, who in the presence of the resident, must attest that the resident is competent, acting voluntarily and is not being coerced. If the resident is residing in a nursing home at the time the request is made, one of the witnesses must be designated by the nursing home. However, the Act does not clearly identify licensed Boarding Homes as being required to provide this designation. Accordingly, Merrill Gardens chooses not to make this designation, and will communicate to the resident why the designation is not being made. In addition, under no circumstances shall one of the witnesses be a relative of the resident by blood, marriage, or adoption or any individual who under any will or by operation of law would be entitled to inherit any portion of the resident's estate.
2. Under no circumstances will the owner, operator, or any employee or agent of the community serve as a witness to any written request by a resident for medication that the resident may self-administer to end his or her life under the Act.
3. The community will conduct an immediate service plan review with the resident and the resident's designated representative or family member to discuss the following:
 - Encouragement to involve the assistance of a hospice agency;
 - Limitations of the community's involvement in all aspects of the assisted suicide process;

- Review of the resident's CPR status and POLST form for any changes;
 - Documentation addressing the assisted suicide request that should be provided to the community;
 - Where the dying process will occur;
 - Who the resident has designated to be present when the ingestion of the medication occurs (community team members will not be allowed under any circumstances to be designated or otherwise present, except for the delivery of comfort care services as outlined in the service plan).
4. Help ensure the resident's service plan is updated to reflect the following:
 - Resident's desire to exercise the right to death with dignity;
 - The name and contact information of who the resident has designated to be present during the dying process (community team members will not be allowed under any circumstances to be designated or otherwise present, except for the delivery of comfort care services as outlined in the service plan);
 - Resident's preferred comfort measures during the dying process.
 5. The community will obtain and maintain, in the resident's file, a copy of the resident's request for medications to end his or her life, signed by two witnesses.
 6. If practicable, obtain and maintain, in the resident's file, a copy of the attending physician's written prescription authorizing dispensing of the lethal dose of medication.
 7. Obtain and maintain, in the resident's file, the consulting physician's written confirmation that the resident is suffering from a terminal condition.
 8. Help ensure that the lethal dose of medication, once obtained, is secured in a locked cabinet in the resident's room at all times. The lethal dose of medication shall only be accessible to the resident, his or her designated representative or family member or attending physician.
 9. Community team members will not take possession of the medication for storage or delivery.
 10. If at any time following the ingestion of the lethal dose of medication, medical treatment is requested by the resident, the community's involvement will be limited to calling 911 on behalf of the resident, or his or her representative or family member.
 11. Under no circumstances will community team members be allowed or required to attend or otherwise be present during and after the ingestion of a lethal dose of medication, except for the delivery of comfort care services as outlined in the service plan.



This image shows a single sheet of white paper with horizontal ruling lines. The lines are evenly spaced and run across the width of the page. There are no margins or other markings on the paper.

Who will assist with Medications if designated Family Member becomes unable to assist?

Emergency Contact Name_____

Emergency Contact Telephone Numbers:

Home Phone _____ Work Phone _____ Cell Phone _____

All Medications must remain on the premises whenever the resident is on the premises.

Should staff become aware that the plan is not being followed; the Resident Care Director or designated staff member will attempt to contact the responsible party and the family member providing the assistance. If staff is unable to contact the responsible party or family member providing assistance in a reasonable time frame to ensure that the resident receives needed services, the staff will assist the resident with medications (or arrange for qualified personnel to administer if needed). Staff will also order medications if necessary (unavailable or inadequate supply) from our preferred pharmacy. If medications require the services of personnel or medications not available at the community, arrangements will be made to obtain those services at the community or another location as practicable.

Signature below indicates understanding and acceptance of the plan and responsibility for payment for any services provided by the community or other qualified providers and/or medications that are ordered from the preferred pharmacy as indicated above.

Resident Signature Date_____

Print Name_____

Responsibility Party Signature Date_____

Print Name_____

Signature of Family Member providing Service Date_____

Print Name_____

Signature of Merrill Gardens Representative Date_____



WA Family Assistance with Treatments

Resident Name _____ Apt. Number _____

Family Member who will assist _____

Contact Numbers for Family Member who is assisting with Treatments.

Home Phone _____ Work Phone _____ Cell _____

Family Members Plan to provide Treatment (i.e. family will change bandage every morning)

Who will assist with Treatment if designated Family Member becomes unable to assist?

Emergency Contact Name _____

Emergency Contact Telephone Numbers:

Home Phone _____ Work Phone _____ Cell Phone _____

Should Merrill Gardens team members become aware that the plan is not being followed; the Assisted Living Supervisor or designated team member will attempt to contact the responsible party and the family member providing the assistance. If Merrill Gardens is unable to contact the responsible party or family member providing assistance in a reasonable time frame to ensure that the resident receives needed services, Merrill Gardens team members will assist the resident with treatments (or arrange for qualified personnel to provide treatment if needed) at the resident's expense.

Signature below indicates understanding and acceptance of the plan and responsibility for payment for any services provided by the community or other qualified provider.

Resident Signature Date _____

Responsibility Party Signature Date _____

Print Name _____

Signature of Family Member providing Service Date _____

Signature of Merrill Gardens Representative Date _____

WA Hazardous Drug Control.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Team members may be exposed to hazardous drugs in the air or on work surfaces, clothing, medical equipment, and resident urine or feces. The health risk depends on how much exposure a worker has to these drugs and how toxic they are. Exposure risks can be greatly reduced by using proper procedures and protective equipment for handling hazardous drugs. Certain activities may be addressed in this policy that are not permitted in Merrill Gardens communities, either by state regulation or policy, but are included as part of NIOSH standards.

Definitions

Hazardous Drugs

Drugs that are known or suspected to cause adverse health effects from exposures in the workplace. They include drugs used for cancer chemotherapy, antiviral drugs, hormones, some bioengineered drugs, and other miscellaneous drugs. For the most current list of hazardous drugs, check the CDC website: www.cdc.gov/niosh/topics/hazdrug/

PPE (Personal Protective Equipment)

Equipment worn to minimize exposure to serious workplace injuries and illnesses. PPE may include items such as gloves, safety glasses or goggles, masks, respirators, and gowns.

Ventilated Containment devices

BSC: Class II biological safety cabinet, CACI: Compounding aseptic containment isolator, CSTD: Closed system transfer device. Activities requiring a ventilated containment device will not be permitted in a Merrill Gardens community.

Guidelines

Activities with the Potential for Exposure

Team members may be exposed to hazardous drugs when they create aerosols, generate dust, clean up spills, or touch contaminated surfaces during the preparation, administration, or disposal of hazardous drugs. The following list of activities may result in exposures through inhalation, skin contact, ingestion, or injection:

- Reconstituting powdered or freeze-dried drugs and further diluting either the re-constituted powder or concentrated liquid forms of hazardous drugs.
- Expelling air from syringes filled with hazardous drugs.
- Administering hazardous drugs by intramuscular, subcutaneous, or intravenous (IV) routes.

- Counting out individual, uncoated oral doses and tablets from multi-dose bottles.
- Crushing tablets to make oral liquid doses.
- Contacting measurable concentrations of drugs present on drug vial exteriors, work surfaces, floors, and final drug products (bottles, bags, cassettes, and syringes).
- Handling contaminated wastes generated at any step of the preparation or administration process of the above.
- Handling unused hazardous drugs or hazardous-drug-contaminated waste.
- Decontaminating and cleaning drug preparation or clinical areas.
- Transporting infectious, chemical, or hazardous waste containers.
- Removing and disposing of personal protective equipment (PPE) after handling hazardous drugs or waste.

Determining Whether a Drug is Hazardous

The “NIOSH List of Antineoplastic and Other Hazardous Drugs in Healthcare Settings, 2014” or subsequent update should be kept in the med room accessible to team members. For the most current list of hazardous drugs, check the CDC website: www.cdc.gov/niosh/topics/hazdrug/

- The Resident Care Director (RCD) is responsible for reviewing all new drug orders to determine whether they are considered hazardous per the NIOSH list and what precautions are to be implemented.
- If a new order is filled before the RCD will be able to review, the assigned med passer on duty when the order is received is to check the list of hazardous drugs and implement any needed precautions. The RCD is to follow up with their own review.
- A Hazard Assessment will be completed for all drugs determined to be hazardous.

Receiving and Storing Hazardous Drugs

All drugs, hazardous or not, will be received and transported in closed containers. Any drugs brought to community staff outside of closed containers will be treated as hazardous drug spills and disposed of accordingly.

STANDARD PRECAUTIONS FOR ALL DRUGS

Standard precautions – all hazardous drugs will be received, transported, handled, assisted with, administered, and disposed of following the standard precaution of wearing gloves. Hazardous drugs meeting this level of hazard will not be specifically labeled.

PRECAUTIONS FOR HAZARDOUS DRUGS

Refer to the table for detailed instructions for handling specific hazardous drug types. When working with hazardous drugs, follow the guidelines listed below:

- Begin exposure control when hazardous drugs enter the community. The most significant exposure risk during distribution and transport is from spills resulting from damaged containers.
- Merrill Gardens will work with pharmacies to mark and label hazardous drugs as follows:

- Yellow – these drugs require double gloves and a gown when received, transported, handled, administered and disposed of.
- Red – these drugs require double gloves, gown, eye and respiratory protection when received, transported, handled, administered and disposed of.
- Prepare for the possibility that spills might occur while team members are handling containers (even when packaging is intact during routine activities) and provide them with appropriate PPE.
- Make sure that medical products have labeling on the outsides of containers that will be understood by all team members who will be separating hazardous from nonhazardous drugs. Wear chemotherapy gloves, protective clothing, and eye protection when opening containers to unpack hazardous drugs to prevent contamination from spreading if damaged containers are found.
- Wear chemotherapy gloves to prevent contamination when transporting the vial or syringe to the work area.
- Store hazardous drugs separately from other drugs.
- Store and transport hazardous drugs in closed containers that minimize the risk of breakage.
- Make sure the storage area has sufficient general exhaust ventilation to dilute and remove any airborne contaminants.

PPE AND ENGINEERING CONTROLS FOR WORKING WITH HAZARDOUS DRUGS

Formulation	Activity	Double Gloves	Protective Gown	Eye Protection	Respiratory Protection	Ventilated engineering controls
Intact capsule or pill	Administration from unit dose packaging	No (single glove should be used)	no	no	no	N/A
Tablets or capsules	Cutting, crushing or otherwise manipulating	yes	yes	no	yes if not done in a control device	yes*
	Administration	yes	yes	no	yes if powder generated	N/A
Oral liquid drug	Administration	yes	yes	no	no	N/A
Topical Drug	Administration	yes	yes	yes, if liquid could splash	yes, if inhalation potential	N/A
Ampoule	Opening	yes	yes	yes, if not done in a control device	yes, if not done in a control device	yes. BSC or CACI*

Sub Q, IM injection	Preparation (withdrawing from vial or ampoule?)	yes	yes	yes, if not done in a control device	yes, if not done in a control device	yes. BSC or CACI*
	Administration from prepared syringe	yes	yes	yes, if liquid that could splash	yes, if inhalation potential	yes. BSC or CACI; recommend use of CSTD*
Solution for irrigation	Administration (bladder, HIPEC, limb perfusion, etc.)	yes	yes	yes	yes	N/A
Powder/ solution for inhalation	Inhalation	yes	yes	yes	yes	yes, when applicable*

*If a ventilated containment system is not available, double gloves, a protective gown, respiratory protection and a disposable pad to protect the work surface should be used.

Drug Preparation and Administration

When preparing and administering hazardous drugs, use these precautions:

- Limit access to areas where drugs are prepared to protect persons not involved in drug preparation.
- Coordinate tasks associated with preparing and administering hazardous drugs for most effective control of worker exposures.
- Wear protective gloves and gowns if you are involved in preparation activities such as opening drug packaging, handling vials or finished products, labeling hazardous drug containers, or disposing of waste.
- Wear PPE (including double gloves and protective gowns) while reconstituting and admixing drugs:
 - Make sure that gloves are labeled as chemotherapy gloves and make sure such information is available on the box or from the manufacturer.
 - Consider using chemotherapy gloves for hazardous drugs that are not chemotherapy drugs or for which no information is available.

Spill Response and Cleanup

All housekeeping, maintenance and resident care team members are responsible for responding to spills of hazardous drugs. Areas of hazardous drug spills will be restricted until the spill is completely cleaned up and the area decontaminated. Spill response kits should be located in the med room.

Spills of known hazardous drugs will be cleaned up according to manufacturer specifications. At a minimum this will include:

- Utilizing the labeled standard precautions.
- Utilizing dedicated spill response kits.

- All surfaces potentially contaminated by known hazardous drugs during preparation or spills will be cleaned with an appropriate solution following standard procedure for general disinfection or as noted by manufacturer recommendations in the hazard analysis.

Spill response kits should contain, at a minimum:

- Deactivation - 2% Sodium Hypochlorite solution or equivalent
- Decontamination – dedicated broom/dustpan/rags
- Cleaning -Tri or Quadra-valent detergent or equivalent
- Disinfection – Isopropyl Alcohol 70% or equivalent

Engineering Controls and Maintenance

All equipment related to this program will be placed on a routine maintenance program to check for proper functioning. Routine maintenance will be of a type and frequency that at a minimum meets manufacturer recommendations; additional type and frequency may be added at the discretion of the General Manager. All maintenance will be performed only after following all control procedures outlined on the hazard assessment form.

Team Member Exposure Determination

Merrill Gardens has determined that all team members in the following jobs may have occupational exposure to hazardous drugs during the normal course of their work:

- Nurses
- Caregivers

The following is a list of job classifications in which some team members at this community may have occupational exposure. Included is a list of tasks and procedures, or groups of closely related tasks and procedures, in which occupational exposure may occur for these individuals:

- Receptionist/Front Desk coverage: Handling drug deliveries, shipping regulated waste.
- Housekeepers: Cleaning hazardous drug handling areas, handling resident trash.
- Maintenance: Repairing hazardous drug handling areas, transporting regulated waste containers for shipping.

Training and Information

The General Manager is responsible for the team member training program. The procedures for how team members will be informed and trained are as follows:

All team members will receive general training on drug hazards during new hire orientation by the Resident Care Director. Job-specific drug safety information will be provided by the Department Head during training. Routine refresher training on hazardous drugs will be provided at least once per year.

The General Manager is responsible for making sure that before starting work, each new employee of this community will attend a health and safety orientation that includes the following:

- An overview of the requirements contained in the Hazardous Drug Control Program
- Hazardous drugs present in the work area with reasonably determined occupational exposure

- Physical and health risks of the hazardous drugs
- The symptoms of exposure
- How to determine the presence or release of hazardous drugs in the work area
- How to reduce or prevent exposure to hazardous drugs through use of control procedures, work practices, and personal protective equipment
- Steps the employer has taken to reduce or prevent exposure to hazardous drugs
- Procedures to follow if team members are exposed to hazardous drugs
- Location of the written hazardous drug control program
- For those tasks or procedures requiring team member use of respiratory protection: team members will receive the necessary training for respiratory protection.
- Those team members who will respond to chemical emergency spills or releases, and will potentially conduct those operations where high levels of exposures to toxic substances could exist or pose a serious danger to team members will receive the appropriate level of training based upon their anticipated duties.

Before introducing a new hazardous drug with reasonably determined occupational exposure into the work area of this community, each affected team member will be given an orientation as outlined above for the new drug.

All training will be appropriately documented and maintained with other team member training records.

Program Responsibilities

The community is to implement a Hazardous Drug Control Program (HDCP). Those team members who are determined to have occupational exposure to hazardous drugs are to comply with the procedures and work practices outlined in this HDCP. The General Manager is the HDCP Administrator and is responsible for:

- HDCP implementation
- HDCP maintenance, review and updates.
- arranging to have necessary personal protective equipment (e.g. gloves, gowns, masks) provided and available in the appropriate sizes.
- verifying necessary engineering controls (e.g. closed system transfer device or controlled area) are properly maintained and used.
- employee HDCP training, which is to be delivered and documented by an adequate person.

The HDCP Administrator may delegate any of the above responsibilities to a designated person.

This document's purpose is only to provide general guidance. It is not a definitive interpretation for how to comply with DOSH requirements. Consult the actual NIOSH hazardous drugs list and program regulations in entirety to understand all specific compliance requirements.

Minimum PPE Required		PPE Requirements Hazardous Drug Handling					Minimum PPE Required
Universal (Green) - handling and disposed of using normal precautions.		Other Drug Name	Handling only - No alteration	Tablet		Open Capsule	Additional Information
Low (Yellow) - handle at all times with gloves and appropriate engineering controls.				Crush/Split	Crush/Split		
Moderate (Orange) - handle at all times with gloves, gown, eye and face protection (with splash potential) and appropriate engineering controls		Dispensed/Common Drug Name (Formulation) and (NIOSH CATEGORY #)	Minimum PPE Required	Minimum PPE required	Minimum PPE required	Minimum PPE required	
abacavir (susp) (2)		ziagen/epzicom/trizivir	Low				
abacavir (tablet) (2)		ziagen/epzicom/trizivir	Universal	Low			
aciclovir (capsule) (3)		soriatane	Universal		Moderate		
anastrozole (tablet) (1)		arimidex	Low	Moderate		Moderate	
androgel (capsule) (3)		methyltestosterone	Universal		High		
apomorphine (inj sq) (2)		apomorphine	Moderate			Moderate	
arthrotec/cytotec (tablet) (3)		diclofenac/misoprostol	Universal	Low	Moderate		
astagraf XL (capsule) (2)		tacrolimus	Universal			do not open	
avodart (capsule) (3)		dutasteride	Universal			Moderate	
azathioprine (tablet) (2)		apomorphine	Universal	Low	Moderate		
bicalutamide (tablet) (1)		casodex	Low	Moderate	High		
brisdelle (capsule) (3)		paroxetine	Universal			Moderate	
caprelsa (tablet) (1)		vandetanib	Low	do not split/crush			
carbamazepine (suspension) (2)		tegretol	Low				
carbamazepine (tablet) (2)		tegretol	Universal	Low	Moderate		
carbamazepine ER (tablet) (2)		carbatrol/equatrol	Universal	do not split/ crush			
clonazepam (tablet) (3)		Klonopin	Universal	Low	Moderate		
clonazepam ODT (tablet) (3)		Klonopin	Universal	do not split/ crush			
colchicine (capsule) (3)		colchicine	Universal			Moderate	
colcrys (tablet) (3)		colchicine	Universal	Low	Moderate		
crizotinib (capsule) (1)		xalkori	Low			do not open	
cyclophosphamide (capsule) (1)		cyclophosphamide	Low			High	
cyclophosphamide (tablet) (1)		cyclophosphamide	Low	Moderate	High		
cyclosporin (solution) (2)		sandimmune/neoral /engraf	Low				
cyclosporine (capsule) (2)		sandimmune/neoral /engraf	Universal			Moderate	

cyclosporine (eye drops) (2)	restasis	Low	Low	Low	Moderate		
didifenac/misoprostol (tablet) (3)	cytotec	Universal	Universal	Low	do not split/ crush		
divalproex (tablet) (2)	depakote	Universal	Universal	do not split/ crush			
divalproex ER (tablet) (2)	depakote er	Universal	Universal	do not split/ crush			
divalproex sprinkle (capsule) (2)	depakote sprinkles	Universal	Universal	do not split/ crush			
ella (tablet) (3)	ulipristal	Universal	Universal	Low	Moderate		Moderate
emcyt (capsule) (1)	estrामustine	Low	Low				High
estradiol (tablet) (2)	estradiol	Universal	Universal	Low	Moderate		
estradiol (transdermal) (2)	estradiol	Low	Low				
estrogen, conjugated (cream) (2)	premarin	Low	Low				
estrogen, conjugated (tablet) (2)	premarin, prem-pro	Universal	Universal	do not split/ crush			
estrogen/progesterone (patch) (2)	climara, combipatch	Low	Low				
estropipate (tablet) (2)	ogen	Universal	Universal	Low	Moderate		
etoposide (capsule) (1)	toposar	Low	Low				High
exemestane (tablet) (1)	aromasin	Low	Low	Moderate	High		
fareston (tablet) (1)	torémifene	Low	Low	Moderate	High		
ferriprox (tablet) (2)	deferiprone	Universal	Universal	Low	Moderate		
fluconazole (injectable) (3)	diffucan	High	High				only if reproductive risk - if not, universal
fluconazole (tablet) (3)	diffucan	Universal	Universal	Low	Moderate		
fluorouracil (cream/solution) (1)	adrucil/carac/efudex	Moderate	Moderate				chemo rated gloves
fluoxymesterone (tablet) (2)	fluoroplex	Universal	Universal	Low	Moderate		
flutamide (capsule) (1)	flutamide	Low	Low				High
fospheytoin (Inj-IM) (2)	cerebyx	Moderate	Moderate				
fospheytoin (Inj-IV) (2)	cerebyx	High	High				
ganciclovir (Inj) (2)	cytovene	High	High				
geodon (injection) (3)	ziprasidone	Moderate	Moderate				
gilenya (capsule) (2)	tingolimod	Universal	Universal				Moderate
gleevec (tablet) (1)	imatinib	Low	Low	do not split/crush			
hexalen (capsule) (1)	altretamine	Low	Low				High
hydroxyurea (capsule) (1)	droxia/hydra	Low	Low				High
icatibant (syringe/sq) (3)	firazyr	Moderate	Moderate				only if reproductive risk - if not, universal
jalyn (capsule) (3)	dutasteride/tamsulosin	Universal	Universal				Moderate
lamivudine/zidovudine (tablet) (2)	combivir	Universal	Universal	Low	Moderate		
leflunomide (tablet) (2)	arava	Universal	Universal	do not split/ crush			
letairis (tablet) (3)	ambrisentan	Universal	Universal	do not split/crush			
letrozole (tablet) (1)	femara	Low	Low	Moderate	High		
leukeran (tablet) (1)	chlorambucil	Low	Low	Moderate	High		
leuprolide (injection) (1)	lupron depot/eligard/lupaneta	Moderate/ High (syringe/vial)	Moderate/ High (syringe/vial)				
lomustine (capsule) (1)	gleostine	Low	Low				High
lysodren (tablet) (1)	mitotane	Low	Low	do not split/crush			
matulane (capsule) (1)	procarbazine	Low	Low				High
medroxyprogesterone (tablet) (2)	provera	Universal	Universal	Low	Moderate		

megestrol (sustained release) (1)	megace	Moderate			
megestrol (tablet) (1)	megace	Low	Moderate	High	
melphalan (tablet) (1)	alkeran	Low	Moderate	High	
mercaptopurine (susp) (1)	purinethol	Moderate			
mercaptopurine (tablet) (1)	purinethol	Low	Moderate	High	
methotrexate (tablet) (1)	rheumatrex	Low	Moderate	High	
methylergonovine (tablet) (3)	methylergonovine	Universal	Low	Moderate	
multiaq (tablet) (3)	dronedarone	Universal	Low	Moderate	
mycophenolate (tablet) (2)	cellcept	Universal	do not split/ crush		
mycophenolic acid (pwd/recon) (2)	mycophenolic acid	Moderate			
myleran (tablet) (1)	busulfan	Low	Moderate	High	
nevirapine (tablet) (2)	viramune	Universal	Low	Moderate	
nevirapine ER (tablet) (2)	viramune XR	Universal	do not split/ crush		
nevirapine Susp (tablet) (2)	viramune	Low			
nexavar (tablet) (1)	sorafenib	Low	Moderate	High	
oxcarbazepine (susp) (2)	trileptal	Low			
oxcarbazepine (tablet) (2)	trileptal	Universal	Low	Moderate	
oxcarbazepine ER (tablet) (2)	oxtellar XR	Universal	do not split/ crush		
panretin (topical) (3)	alitretinoin	Low			
paroxetine (tablet) (3)	paxil/pexeva	Universal	Low	Moderate	
paxil (suspension) (3)	paroxetine	Low			
paxil CR (tablet) (3)	paroxetine	Universal	do not crush/ split		
phenoxybenzamine (capsule) (2)	dibenzylamine	Universal			moderate
phenytoin (suspension) (2)	dilantin	Low			
phenytoin (tablet) (2)	dilantin	Universal	Low	Moderate	
phenytoin ER (capsule) (2)	dilantin/phenytek	Universal			do not open
progesterone (capsule) (2)	prometrium	Universal			do not open
progesterone (intravaginal) (2)	first progest	Low			
propylthiouracil (tablet) (2)	propylthiouracil	Universal	Low	Moderate	
propylthiouracil (tablet) (2)	propylthiouracil	Universal	Low	Moderate	
qudexy XR (capsule) (3)	propicia/proscar	Universal			
raloxifene (tablet) (2)	topiramate	Universal	do not split/ crush		Moderate
rapamune (solution) (2)	evista	Universal			
rebetal (solution) (3)	sirolimus	Low			
revlimid (capsule) (2)	ribavirin	Low			
ribavirin (capsule) (3)	lenalidomide	Universal			do not open
	ribasphere/moderiba/ copegus/rebetol	Universal			do not open
ribavirin (tablet) (3)		Universal	do not crush/ split		
risperdal (solution) (2)	risperidone	Low			
risperdal consta (injection) (2)	risperidone	Moderate			
risperdal M (dissolve tablet) (2)	risperidone	Universal	do not split/ crush		
risperidone (tablet) (2)	risperdal	Universal	Low	Moderate	
sabril (pwd packet) (3)	vigabatrin	Moderate			
sabril (tablet) (3)	vigabatrin	Universal	Low	Moderate	

sirolimus (tablet) (2)	rapamune	Universal	do not split/ crush		
spironolactone (tablet) (2)	aldactone	Universal	Low	Moderate	
sprycel (tablet) (1)	dasatinib	Low	do not split/crush		
sutent (capsule) (1)	sunitinib	Low			High
tabloid (tablet) (1)	thioguanine	Low	Moderate	High	
tacrolimus (capsule) (2)	prograf	Universal			Moderate
tacrolimus (topical) (2)	protopic	Low			
tamoxifen (tablet) (1)	nolvadex/solhamox	Low	Moderate	High	
tarceva (tablet) (1)	erlotinib	Low	Moderate	High	
targretin (capsule) (1)	bexarotene	Low			High
targretin (gel) (1)	bexarotene	Moderate			chemo rated gloves
tasigna (capsule) (1)	nilotinib	Low			High
temodar (capsule) (1)	temozolomide	Low			High
testosterone (inj- IM) (3)	testosterone	Moderate			only if reproductive risk - if not, universal
testosterone (transdermal- all forms) (3)	striant/testim/androgel androderm/fortesta	Low			
testred (capsule) (3)	methyltestosterone	Universal			Moderate
topiramate (tablet) (3)	topamax	Universal	Low	Moderate	
topiramate sprinkle (capsule) (3)	topamax	Universal			Moderate
topotecan (capsule) (1)	hycamtin	Low			do not open
Tracleer (tablet) (3)	bosentan	Universal	do not split/crush		
tretinoin (gel/cream) (3)	retin-A/renova/	Low			
tronkendi XR (capsule) (3)	topiramate	Universal			do not open
valcyte (sol - reconstitute) (2)	valganciclovir	Moderate			
valganciclovir (tablet) (2)	valcyte	Universal	do not split/ crush		
valproic acid (capsule) (3)	depakene	Universal			Moderate
valproic acid (syrup) (3)	depakane	Low			
Victoza (inj) (2)	liraglutide recombinant	Moderate			
voriconazole (IV) (3)	vfend	High			only if reproductive risk - if not, universal
voriconazole (tablet) (3)	vfend	Universal	Low	Moderate	
votrient (tablet) (1)	pazopanib	Low	do not split/crush		
warfarin (tablet) (3)	coumadin	Universal	Low	Moderate	
xeloda (tablet) (1)	capecitabine	Low	Moderate	High	
zelboraf (tablet) (1)	vemurafenib	Low	do not split/crush		
zidovudine (capsule) (2)	retrovir	Universal			Moderate
zidovudine (solution) (2)	retrovir	Low			
zidovudine (tablet) (2)	retrovir	Universal	Low	Moderate	
ziprasidone (capsule) (3)	geodon	Universal			Moderate
zirgan (Eye Gel) (2)	ganciclovir	Low			
zolinza (capsule) (1)	vorinostat	Low			High
zonisamide (capsule) (3)	zonegran	Universal			Moderate
zyflga (tablet) (1)	abiraterone	Low	do not split/crush		

WA Intermittent Nursing Services

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Residents occasionally have needs that require the services of a licensed nurse to perform. While the provision of nursing care is not the level of service generally offered by a Merrill Gardens community, there may be circumstances when it is appropriate for licensed community staff to provide nursing services in a limited capacity. The community may choose to offer intermittent nursing services, or not. Communities contracted for assisted living through the Medicaid/COPES program will offer intermittent nursing services as required.

Definitions

Intermittent Nursing Services

Nursing care, consistent with chapters 18.78 and 18.88 RCW & 246-840 WAC, provided by the community, which does not include continuous skilled nursing care and supervision of the type provided in nursing homes.

Supervision

Review of an LPN'S assessments, oversight and guidance of the nursing component of the Service Plan and Negotiated Service Agreement by a Registered Nurse (RN). The RN does not need to be on the premises during the provision of Limited Nursing Services.

Continuous Skilled Nursing Care and Supervision

Requiring nursing care, monitoring or assessment throughout the course of the day and night, due to complex medical needs or unstable condition.

Guidelines

Intermittent Nursing Services Provided

Intermittent nursing services will only be provided by a Registered Nurse (RN), or a Licensed Practical Nurse (LPN) under the supervision of an RN. Nurse Delegation is not permitted unless approved by the Vice President of Operations.

The following nursing services may be provided:

- Any other nursing service requested by the community and approved in writing by DSHS.

Nursing Services Not Allowed

The following nursing services will not be provided by community staff:

- Respiratory ventilation;
- Intravenous procedures;

- Suctioning;
- Feeding tube insertion or site maintenance;
- Care of residents who are bed-bound for more than 14 consecutive days as a result of a medical condition.
- Insertion of urethral catheters, including indwelling

Residents with these needs may provide self-care or arrange for independent nursing or health care services.

Registered Nurse Requirements

If a community provides Intermittent Nursing Services, they must either employ or contract directly or indirectly with an RN or physician to:

1. Provide or supervise Intermittent Nursing Services
2. Develop and implement systems that support and promote the safe practice of nursing including:
 - Nursing services supervision
 - Initial & ongoing assessments of the nursing needs of each resident
 - Development and amendments to the nursing component of the service plan
 - Implementation of the nursing component of the service plan
 - Availability of the supervisor in person, by phone or pager to respond to the residents needs
3. Assess, or supervise an LPN'S assessment of each resident needing Intermittent Nursing Services upon move-in and develop the nursing component of the resident's negotiated service agreement.
4. Reassess, or supervise an LPN'S reassessment of the resident's nursing needs when staff notices a change in the resident's functional ability or health status and update the nursing component of the resident's negotiated service agreement accordingly.
5. Be available in person, by pager or by telephone during the hours of Intermittent Nursing Services.

Physical Plant Requirements

Communities providing Intermittent Nursing Services will provide for safe and sanitary:

- Storage and handling of clean and sterile nursing equipment and supplies;
- Storage and handling of soiled laundry and linens;
- Cleaning and disinfecting soiled equipment;
- Refuse and infectious waste disposal;

The following should be accessible only by staff:

- A clean utility area for the purposes of storing and preparing clean and sterile nursing supplies, equipped with:
 - A work counter or table, and
 - Adjacent hand-washing sink, with soap and paper towels.
- A soiled utility area for the purposes of storing soiled linen, cleaning and disinfecting soiled nursing care equipment, and disposing of refuse and infectious waste, equipped with:
 - A work counter or table, and
 - Sinks for hand-washing and cleaning/sanitizing, with soap and paper towels

WA Nurse Delegation (Washington only)

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Designated Merrill Gardens communities may provide Nurse Delegation of specific tasks to willing and prepared staff (Nursing Assistant-Certified and Nursing Assistant- Registered) as identified by regulation WAC 246-840.

Procedure

Follow the steps below for Nurse Delegation in the state of Washington:

1. Residents or their representatives will have the Nurse Delegation concept discussed with them prior to starting or receiving Assisted Living services and will sign a general Consent for Nurse Delegation in their Residency Agreement. This form will be kept in the Business Office Record with the Residency Agreement and shall not be thinned from the record.
2. A Registered Nurse may delegate tasks of nursing care when the Resident Care Director/Garden House Director or designee determines that it is in the best interest of the resident for a non-licensed person to provide the task.
3. The Delegating Nurse will:
 - a. Delegate tasks within the RN's scope of practice;
 - b. Determine that the resident:
 - is in a stable and predictable health condition (stable and predictable includes residents on Hospice whose unstable condition is predictable) AND
 - cannot be taught to do the task themselves. Resident independence is to be a primary concern.
 - c. Complete Nurse Delegation Visit form.
 - d. Document note in the Health Record why the resident is a candidate for Nurse Delegation.
4. The Delegating Nurse will determine competency of the team member(s) providing the delegated task by:
 - a. Verifying that team members have completed the Nurse Delegation Training by viewing the certificates in the employee file; AND
 - b. Reviewing the current NAC credential in the employee file; AND

- c. Teaching the task to qualified team members for each resident according to community procedures.; AND
 - d. Individualizing the teaching per resident as needed.
5. The RN will design a clear Delegation Plan for each task. This Delegation Plan will be referred to in the Service Plan in the Medication section. The Delegation Plan will be placed in a plastic sleeve with the resident's current MAR.
 6. The Resident Assistant who is delegated these tasks either through assumption or direct delegation will have their name on the administration procedure form(s). When they initial next to their name they are accepting responsibility and consent for performing the delegated task.
 7. The RN will review how the task is being provided every 90 days by checking with the team member(s) and with the resident/resident's representative. This will be documented on the back of the Delegation Plan.
 8. The RN can modify and re-date the Delegation Plan if there are no significant changes to the plan. (i.e. discontinuing or adding medications with no different method of administering them).
 9. When the Delegation Plan is no longer in effect (medication is discontinued), the form will be removed from the MAR and placed in the resident's Health Record in the Medication section.
 10. The RN may rescind delegation when the resident's condition is no longer stable or predictable. This should be documented on the Rescinding Delegation form.
 11. When the RN is not available or on vacation, he/she must transfer delegation to another RN who will cover the delegated tasks. This transfer is noted in the health record.
 12. Documentation of task completion will be done on the resident's MAR by the delegated Caregiver administering medication. If the task is not provided, their initials will be circled and noted why the task was omitted on the back of the MAR. Any unusual response to the task (e.g. vomiting, additional redness, etc.) will be written in the Communication Log as well as making contact with the Delegating Nurse or the Hospice Nurse or the Supervisor, as indicated on the Delegation Plan.
 13. The Delegating Nurse will keep a list of all residents who have Delegated Tasks and the date for review on the site available for review.

provider is not yet aware of, or needs more information about the POLST form, please have them contact the Washington State Medical Association at 1 (800) 552-0612.

Organizations that Endorse the Use of the POLST Form

Association of Washington Public
Hospital Districts
Washington State Department of Health
Washington State Hospice & Palliative
Care Organization
Washington State Hospital Association
Washington State Medical Association
Regional Ethics Network of Eastern
Washington and Northern Idaho

More information about the POLST form can be found at the Washington State Medical Association website at www.wsma.org/patient_resources/polst.

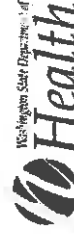
Physician Orders for Life-Sustaining Treatment

(POLST) Form

Information for patients and family members

**Washington
State Medical
Association** 
Physician Driven
Patient Focused

2033 6th Avenue Suite 1100
Seattle, WA 98121
(206) 441-9762 or 1-800-552-0612



Department of Health
Office of Emergency Medical & Trauma Prevention
P.O. Box 47853
Olympia, WA 98504-7853
(360) 236-2828 or (800) 458-5281

he POLST form is intended for any individual with an advanced life-limiting illness.

If you have a serious health condition, you need to make decisions about life-sustaining treatment. Your physician can use the POLST form to represent your wishes as clear and specific medical orders.

Your physician may use the POLST form to write orders that indicate what types of life-sustaining treatment you want or do not want at the end of life.

The POLST form asks for information about:

- your preferences for resuscitation
- medical conditions
- the use of antibiotics
- artificially administered fluids and nutrition.

The POLST form is voluntary and is intended to:

- help you and your physician discuss and develop plans to reflect your wishes;
- assist physicians, nurses, health care facilities and emergency personnel in honoring your wishes for life-sustaining treatment;
- direct appropriate treatment by Emergency Medical Services personnel.

FREQUENTLY ASKED QUESTIONS REGARDING THE POLST FORM

Does the POLST form need to be signed?

Yes. A physician, nurse practitioner, or certified physician assistant (PA-C) must sign the bright green form in order for it to be a physician order that is understood and followed by other health care professionals.

If I have a POLST form do I need an advanced directive too?

If you have a signed POLST form, it is recommended that you also have an advanced directive, though it is not required. You may obtain more information about advanced directives from your physician.

What if my loved one can no longer communicate his/her wishes for care?

If you are the designated health care representative, you can speak on behalf of your loved one. A physician can complete the POLST form based on your understanding of your loved one's wishes.

In what setting is the POLST form used?

The completed POLST form is a physician order form that will remain with you if you are transported between care settings, regardless of whether you are in the hospital, at home or in a long-term care facility.

Where is the POLST form kept?

If you live at home you should keep the original bright green POLST form in a prominent location (e.g., on the front of the refrigerator, on the back of the bedroom door, on a bedside table, or in your medicine cabinet). If you reside in a long-term facility, your POLST form may be kept in your medical chart along with other medical orders.

How do I obtain a copy of the POLST form?

From your physician or other health care provider. If your physician or other health care

HIPAA PERMIT'S DISCLOSURE OF POLST TO OTHER HEALTH CARE PROVIDERS AS NECESSARY

Physician Orders for Life-Sustaining Treatment

FIRST follow these orders. **THEN** contact physician, nurse practitioner or PA-C. The POLST is a set of medical orders intended to guide emergency medical personnel in the treatment of persons with advanced illness based on their current medical condition and goals. Any section not completed implies full treatment for that section. Everyone shall be treated with dignity and respect.

Last Name - First Name - Middle Initial
Date of Birth - Last 4 SSN - Gender - M - F
Medical Conditions/Patient Goals
Agency Info/Sucker

A CARDIOPULMONARY RESUSCITATION (CPR): Patient has no pulse and is not breathing.

☐ CPR/Attempt Resuscitation ☐ DNR/Do Not Attempt Resuscitation (Allow Natural Death)
Choosing DNR will include appropriate comfort measures and may still include the range of treatments below. When not in cardiopulmonary arrest, go to part B.

B MEDICAL INTERVENTIONS: Patient has pulse and/or is breathing.

COMFORT MEASURES ONLY: Use medication by any route, positioning, wound care and other measures to relieve pain and suffering. Use oxygen, oral suction and manual treatment of airway obstruction as needed for comfort. **Patient prefers no hospital transfer. EMS contact medical control to determine if transport indicated to provide adequate comfort.**

☐ **LIMITED ADDITIONAL INTERVENTIONS:** Includes care described above. Use medical treatment, IV fluids and cardiac monitor as indicated. Do not use intubation or mechanical ventilation. May use less invasive airway support (e.g. CPAP, BiPAP). **Transfer to hospital if indicated. Avoid intensive care if possible.**

☐ **FULL TREATMENT:** Includes care described above. Use intubation, advanced airway interventions, mechanical ventilation, and cardioversion as indicated. **Transfer to hospital if indicated. Includes intensive care.**

Additional Orders: (e.g. dialysis, etc.)

C SIGNATURES:

The signatures below verify that these orders are consistent with the patient's medical condition, known preferences and best known information. If signed by a surrogate, the patient must be decisionally incapable and the person signing is the legal surrogate.

Discussed with: ☐ Patient ☐ Parent of Minor ☐ Health Care Agent ☐ Legal Guardian (DPOAC) ☐ Spouse/Other

PRINT -- Physician/ARNP/PA-C Name
PRINT -- Patient or Legal Surrogate Name
PRINT -- Patient or Legal Surrogate Signature (mandatory)

Phone Number
Date
Phone Number
Date